

NURSING CARE REPORT

Patient's Name: TERESIA WANGECHI

ID/Passport No.: N/A

Patient Number: 318543

Phone No.: 254790218335

Age: 32 yrs 4 months

Postal Address: N/A

Admitted On: Tue, 21 Apr 2026 05:41 AM

Village: GITHURAI 44

36 CARDEX 52 VITALS MONITOR 1 FLUID BALANCE CHART

Entry By	Entry Date	Note
Tue, 21 Apr 2026		
Anthony Wamalwa Barasa	Tue, 21 Apr 2026 at 05:46	PATIENT WAS BROUGHT TO THE ER UNCONSCIOUS WITH A 1 DAY HISTORY OF PV BLEEDING breathing spontaneously at room air GCS 8/15 PATIENT BROUGHT WITH HX OF PV BLEEDING REPORTS SHE has been spotting reports her pregnancy was approximately 8 weeks RELATIVE REPORTS PATIENT STARTED BLEEDING 1 DAY AGO WITH CLOTS. SHE IS ALSO SAID TO BE 8/40 BUT NOT C ONFIRMED BY SCAN. FOR THE ABOVE SYMPTOMS SHE WENT TO A PERIPHERAL FACILITY WHERE SHE WAS REFERRED TO OUR FACILITY FOR PELVIC ULTRASOUND. NOTED TO HAVE 1 EPISODE OF GTC. informed shift managed of the patient condition blood work ups done cross match blood group Patient noted to have low BPS PATIENT IS PARA 3 + 0 GRAVIDA HAS previous hx of c/s in appx 2013 patient has no known food or drug allergies has no underlying comorbidity as reported b the informant the spouse patient cannulated administered iv voluven 50mls stat iv Keppra 500mg stat patient transfused 600mls of whole blood stat administered iv phenytoin 1 gm stat SPECULUM DONE SOME CLOTS EXPELLED catheterization done minimal urine collected in urine bag PATIENT BOTH BILATERAL LOWER LIMB AND UPPER LIMBS COLD COVER WARM HEATER PROVIDED PATIENT ATIRE CHANGED TO HOSPITAL GOWN PATIENT HAS AN OBVIOUS SCAR AT THE PUBIC REGION relative reports patient smokes bang and take alcohol patient is a business lady at Kimbo patient relative report patient is a Christian PATIENT HAS ACTIVE bowel and bladder elimination HOSP ADMIN INFORMED BY THE SHIFT MANAGER ON PT STATUS PATIENT RELATIVE APPRAISED ON THE NEED FOR MVA CONSENTED FOR THE PREOCEDURE THEATER STAFF appraised on patient condition PATIENT WHEELED TO THEATER handed over to theater staff
Lucy Wambui Ndirangu	Tue, 21 Apr 2026 at 08:07	5.55am Received the patient in theatre escorted by ER nurses for MVA Patient came in unconscious, breathing spontaneously in room air saturating at 98% had a GCS of 10/15, only responding to pain she had a scan done that showed incomplete miscarriage at 8/40 patient with pv bleeding had an pack in situ which was fully soaked had convulsed several times, phenytoin running from opd, received 1 pint of blood in ER She is a para 3 gravida 4 consent signed, tagged, cannulated, has a catheter in situ vitals bp 113/71mmhg, pulse 78b/min spo2 98%, resp 18b/min 6.08am patient escorted into the operating room, placed on the operating table, connected to the cardiac monitor vitals bp 113/76mmhg, pulse 80b/min spo2 100% general anesthesia administered, patient positioned lithotomy and the lower limbs secured relative appraised on the need for hdu care after the procedure patient cleaned with hibitane and iodine and draped aseptically 6.45am time out done according to WHO, the procedure began vitals bp 103/54mmhg, pulse 90b/min spo2 99% pocs extracted, patient still bleeding 2 pints of ffps available and confirmed ffp transfusion began no adverse reactions noted vitals bp77/45mmhg, pulse 80b/min spo2 100% 7.00am procedure ongoing no concerns noted, pocs collected and taken to the lab vitals bp 77/38mmhg, pulse 88b/min spo2 100% 7.15am procedure on progress 2nd pint ffp commenced vitals bp 74/35mmhg, pulse 82b/min spo2 100% 7.25am procedure ended successfully all the pocs extracted p/r misoprostol 800mcg administered, patient cleaned pad placed to monitor bleeding patient handed over to the anesthetist for reversal vitals bp 83/53mmhg, pulse 81b/min spo2 99% 7.40am patient reversed successfully, extubated vitals bp 143/88mmhg, pulse 78b/min spo2 100% handed over to HDU for further monitoring
Purity Mwende Mulwa	Tue, 21 Apr 2026 at 09:00	patient received from theatre staffs sick looking, drowsy but arousable and responding to voice and obeying commands GCS- 10/15 breathing spontaneously in room air and saturating well with no signs of respiratory distress noted para 3+1 doing day 0 post MVA secondary to incomplete miscarriage patient was admitted with hypovolemic shock, history of massive bleeding for 6hrs, convulsion ? cause she also reports to have been spotting for 1 week MVA done, drained approximately 200cc of RPOC Has received 3 pints of whole blood and 2 pints of FFPs, Preop Hb 8.5 Had 1 episode of GTC lasting less than 5 min Received Phenytoin loading dose MVA done under GA(Ketamine, Midazolam) plan of care 1.RPT FHG, UECS, do extended electrolytes (Ca2+, Mg2+) 2. RPT scan 3. Monitor vitals and urine output 4. Hold transfusion for now until rpt Hb 5. Monitor vitals until fully awake 6. Can ambulate and feed once fully awake 7. OBGyn review 8. CT antibiotics per T-sheet 9. IV maintainance fluids- DNS/RL 2L in 24 hrs (84cc/hr)

Purity Mwende Mulwa	Tue, 21 Apr 2026 at 10:30	hygiene needs met, bed bath done, diaper changed, bed linen changed and patient changed into a clean gown patient left on bed stable, still drowsy and sedated BP- 93/60mmhg PR- 90 b/min SPO2- 97% RR- 16b/min ct care
Purity Mwende Mulwa	Tue, 21 Apr 2026 at 11:30	Plan in D/w Dr Misbah 1. Do the rpt scan 2. Do rpt Hb in the afternoon 3. Don't give too much fluids, encourage oral rehydration once fully awake, give slow infusion of 1L 4. Don't give more anticonvulsant, assess first, CT antibiotics 5. Review with results
Purity Mwende Mulwa	Tue, 21 Apr 2026 at 11:50	patient in the ward still drowsy BP- 102/76 PR- 76 spo2- 99 temp- 36.4 left on bed asleep ct care
Purity Mwende Mulwa	Tue, 21 Apr 2026 at 12:30	patient fully awake but declined taking oral sips and not responding to any questions
Purity Mwende Mulwa	Tue, 21 Apr 2026 at 13:30	patient agreed to take oral sips served with a cup of warm water, took all and retained declined blended food ct care
Purity Mwende Mulwa	Tue, 21 Apr 2026 at 14:30	due treatment administered as per t.sheet vital signs taken and recorded ct care
Purity Mwende Mulwa	Tue, 21 Apr 2026 at 16:15	anaestheologist review done patient fully awake, well oriented to time, place patient reports that she feels that she may die after a few hour since patients admitted in ICU don't live for long patient reassured but she is psychologically disturbed Plan in D/w Dr Njihia 1. Deescalate to the ward- maternity team informed to prepare 2. Psychosocial support 3. Suicidal watch
Purity Mwende Mulwa	Tue, 21 Apr 2026 at 18:00	due treatment administered as per tsheet vital signs taken and recorded ct care
Purity Mwende Mulwa	Tue, 21 Apr 2026 at 18:10	patient fully stable in the ward no concerns raised transferred to obs gyn ward and settled on bed ct care
Purity Mwende Mulwa	Tue, 21 Apr 2026 at 18:40	patient handed over to dayshift staffs in a FGC awake, alert and conscious well oriented to time, place and person GCS- 15/15 breathing spontaneously in room air and saturating well with no signs of respiratory distress noted para 3+1 doing day 0 post MVA secondary to incomplete miscarriage patient was admitted with hypovolemic shock, history of massive bleeding for 6hrs, convulsion ? cause she also reports to have been spotting for 1 week MVA done, drained approximately 200cc of RPOC Has received 3 pints of whole blood and 2 pints of FFP patient is having suicidal thoughts, has been counselled and still insists on committing suicide- for close monitoring feeds well on light diet and retains has not opened bowels nor passed flatus since the procedure was done but empties bladder via a urinary catheter in situ currently non ambulant- reports that her body is still weak plan of care -monitor vital signs -close monitoring -psychosocial support ct care
Lydia Mandera Ongwae	Tue, 21 Apr 2026 at 19:30	Received the patient from dayshift staffs awake, alert and conscious, breathing spontaneously at room air with no signs of respiratory distress noted she is well oriented to time, place and person with a GCS- 15/15 she is a para 3+1 doing day 0 post MVA secondary to incomplete miscarriage, drained approximately 200cc of RPOC patient was admitted with hypovolemic shock, history of massive bleeding for 6hrs, she also reports to have been spotting for 1 week She has received 3 pints of whole blood and 2 pints of FFP patient is having suicidal thought sand was counselled over the same. PLAN For close monitoring medication as per the tsheet. ct care
Lydia Mandera Ongwae	Tue, 21 Apr 2026 at 21:00	Counselling session offered to both the patient and her husband and they admitted that they are going through a tough times in their marriage that has lasted for the past 10yrs. More sessions to be booked so that they can benefit from them.

Lydia Mandera Ongwae	Tue, 21 Apr 2026 at 22:35	Patient received her due treatment and tolerated well.
Wed, 22 Apr 2026		
Lydia Mandera Ongwae	Wed, 22 Apr 2026 at 00:15	Patient ambulated successfully to the toilet and opened bowel well with nil complain
Lydia Mandera Ongwae	Wed, 22 Apr 2026 at 02:30	Patient sleeping calmly in bed.ct monitoring closely.
Lydia Mandera Ongwae	Wed, 22 Apr 2026 at 04:30	Patient asleep.
Lydia Mandera Ongwae	Wed, 22 Apr 2026 at 06:00	patient received her due treatment metronidazole 500mg i.v and ceftriaxone 1gm i.v and tolerated well.
Damaris Nyambeki Mauti	Wed, 22 Apr 2026 at 08:00	patient handed over to day shift staffs in a FGC awake, alert and conscious she is well oriented to time, place and person GCS- 15/15 breathing spontaneously in room air and saturating well with no signs of respiratory distress noted para 3+1 doing day 1 post MVA secondary to incomplete miscarriage patient was admitted with hypovolemic shock, history of massive bleeding for 6hrs, convulsion ? cause she also reports to have been spotting for 1 week Has received 3 pints of whole blood and 2 pints of FFP post yesterday patient is having suicidal thoughts, has been counselled and still insists on committing suicide- psychosocial counselling was done at night with both partners presence feeds well on light diet and retains has not opened bowels nor passed flatus since the procedure was done but empties bladder via a urinary catheter in situ for close monitoring plan of care -monitor vital signs -close monitoring -psychosocial support -ct care
Purity Mwende Mulwa	Wed, 22 Apr 2026 at 08:20	patient received from night shift staffs in a FGC awake, alert and conscious she is well oriented to time, place and person GCS- 15/15 breathing spontaneously in room air and saturating well with no signs of respiratory distress noted para 3+1 doing day 1 post MVA secondary to incomplete miscarriage patient was admitted with hypovolemic shock, history of massive bleeding for 6hrs, convulsion ? cause she also reports to have been spotting for 1 week Has received 3 pints of whole blood and 2 pints of FFP post yesterday patient is having suicidal thoughts, has been counselled and still insists on committing suicide- psychosocial counselling was done at night with both partners present feeds well on light diet and retains has not opened bowels nor passed flatus since the procedure was done but empties bladder via a urinary catheter in situ for close monitoring plan of care -monitor vital signs -close monitoring -psychosocial support -ct care
Purity Mwende Mulwa	Wed, 22 Apr 2026 at 08:30	hygiene needs met, bed making done and patient changed into clean gown
Purity Mwende Mulwa	Wed, 22 Apr 2026 at 09:00	DWR Repeat TBC today Encourage ambulation Remove urethral catheter Hold IV fluids T-sheet as revised
Purity Mwende Mulwa	Wed, 22 Apr 2026 at 10:30	patient served with breakfast, took all and retained
Purity Mwende Mulwa	Wed, 22 Apr 2026 at 14:00	due treatment administered as per tsheet vital signs taken and recorded ct care

Phaustine Atamba Lung'atso	Wed, 22 Apr 2026 at 14:42	Patient reviewed by gynae, patient doing well Concerns of her psychological well being addressed Repeat HB 9.1gdl Patient is vitally stable plan of care encourage continuous psychosocial care Discharge home through Psychiatric clinic
Phaustine Atamba Lung'atso	Wed, 22 Apr 2026 at 15:53	Patient appraised on discharge, discharge summary filled, iv meds returned to pharmacy Patient awaits clearing from from billing office
Purity Mwende Mulwa	Wed, 22 Apr 2026 at 17:00	PATIENT SERVED WITH PORRIDGE, TOOK ALL AND RETAINED
Purity Mwende Mulwa	Wed, 22 Apr 2026 at 18:30	PATIENT IN THE WARD STABLE SAID that even if we hold her for years she will never be able to clear her bill and we are holding her against her will and that we are chasing away the only people who are able to pay her bill nurse covering informed about the case handed over to night duty staff for continuity of care
Damaris Nyambeki Mauti	Wed, 22 Apr 2026 at 19:00	patient received in the unit from day duty staff in bed awake alert and conscious lying in bed while eating- she said its comfortable there she is well oriented to all aspects of time place nad person breathing spontaneously on room air no sign of respiratory distress she is doing ady 2 post MVA due to incomplete miscarriage patient who was admitted in shock due to excessive PV bleeding for 6hrs at home, she is currently stable post MVA Today she was discharged through psychiatry , relatives informed about the discharge, they have not yet cleared the patient has reported early to the day shift staff that she needs to go home, she is being held here against her wish patient was explained to that its the the policy of the hospital and their responsibility to clear the hospital bill before the are cleared to go home the patient has psycho-social issues and keeps taking of suicidal attempts plan monitor vital signs close monitoring for any suicidal attempt ct with the discharge meds
Damaris Nyambeki Mauti	Wed, 22 Apr 2026 at 19:45	patient took her supper and she retained , no concern reported
Damaris Nyambeki Mauti	Wed, 22 Apr 2026 at 22:15	patient reported that her legs are so cold she was given extra blanket vital signs were taken and charted due medications of po flomax 500mg, po betapyn 1 tab po were administered and retained
Thu, 23 Apr 2026		
Damaris Nyambeki Mauti	Thu, 23 Apr 2026 at 02:18	patient is asleep, normal resps noted, she is left undisturbed
Damaris Nyambeki Mauti	Thu, 23 Apr 2026 at 06:15	vital signs were taken and charted due medications of po cefuroxime 500mg, po betapyn 1 tab, po flamox 500mg were administered and retained
Damaris Nyambeki Mauti	Thu, 23 Apr 2026 at 08:00	patient handed over to day duty staff in bed awake alert and conscious she is well oriented to all aspects of time place nad person breathing spontaneously on room air no sign of respiratory distress she is doing ady 3 post MVA due to incomplete miscarriage patient who was admitted in shock due to excessive PV bleeding for 6hrs at home, she is currently stable post MVA she was discharged yesterday through psychiatry , she remains as a discharge in in the unit , relatives informed about the discharge, they have not yet cleared the patient has reported yesterday to the day shift staff that she needs to go home, she is being held here against her wish patient was explained to that its the the policy of the hospital and their responsibility to clear the hospital bill before the are cleared to go home the patient has psycho-social issues and keeps taking of suicidal attempts plan monitor vital signs close monitoring for any suicidal attempt ct with the discharge meds

21-Apr-26

OP/IP NO: 419390

REF DOC: ICU

WARD

EXAMINATION: PELVIC ULTRASOUND

Clinical. Info: post MVA 2⁰ to incomplete abortion at 8/40

Technique: Convex probe on B and C Modes

Findings;

The uterus is anteverted, normal in size, measuring 9.05cm × 7.02cm × 5.07cm with a calculated volume of 168.41 cc. Myometrial echotexture is homogeneous with no focal lesions identified.

An echogenic collection with areas of anechoic free fluid is noted in the endometrial cavity measuring a volume of 27.08ml in-keeping with RPOCs.

Both adnexa are clear.

Cervix is homogeneous.

No free fluid collections noted in POD or pelvic cavity.

Well-distended and unremarkable.

OPINION:

- *RPOCs as described above.*

RADIOSONOGRAPHER
CAROLYNE

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